HEALTH SERVICES ADMINISTRATION PORTFOLIO · OCTOBER 2026

Keeping rural care open: Prairie Ridge Community Hospital

A health policy case study on rural hospital viability under the 2026 to 2028 Medicaid, ACA and Medicare changes, with the supporting analysis, model and board materials.

Project case study 2–21
Executive summary, regulatory frameworks, mock data and interventions, outcomes, AI approach and the visual asset list.

Board decision memo 22–26
Recommendation, dated policy changes, options compared, budget, triggers, gates and the first 90 days.

Executive slide deck 27–36
Ten slides from the problem to the three board decisions.

KPI dashboard 37–38
Scenario margins, result bridge, initiatives, headwinds, sensitivity, triggers, gates and AI returns.

Scenario model (Excel) 39–46
Printout of the 8-tab workbook: inputs, baseline, headwinds, initiatives, scenarios, sensitivity and AI return.

The finding. Holding course as a Critical Access Hospital leaves a −7.8% margin by FY2028. Converting to a Rural Emergency Hospital reaches +3.0% but ends inpatient care. A CAH transformation reaches +0.9%, $2.55M a year better than holding, with REH kept as a trigger-based fallback.

Prairie Ridge is a fictional 25-bed hospital; every hospital figure is an illustrative model assumption. Policy facts reflect sources checked in October 2026: CMS MLN Fact Sheet MLN2259384 (REH payment), Jones Day on the No Surprises Act dispute rule, and KFF Health News on AI in rural care.

Rural Hospital Viability in a Shifting Payment Landscape: A Health Policy Case Study

Oct 25, 2026 · @Neha

1. Executive Summary & Problem Statement

Recommendation: keep Prairie Ridge Community Hospital a Critical Access Hospital (CAH), fund a three-part transformation plan, and pre-authorize a Rural Emergency Hospital (REH) conversion if defined financial triggers are hit. In this illustrative model, that path moves FY2028 operating margin from −7.8% under the status quo to +0.9%, while keeping inpatient care in the community.

Scope note: Prairie Ridge is a fictional hospital. Every hospital-level figure below is an illustrative assumption built for this exercise. Policy facts (payment amounts, rule dates, program funding) come from the sources listed in Section 6 and are current as of October 25, 2026.

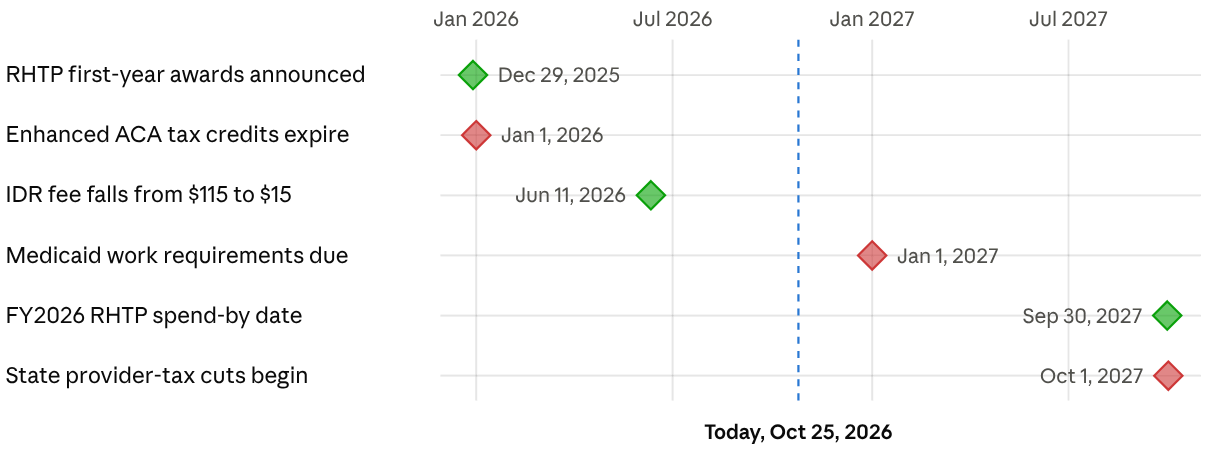
The problem

Federal policy is changing three things for small rural hospitals at the same time. Coverage is shrinking, Medicaid financing is tightening, and new payment and grant options are opening.

- **Coverage:** the enhanced ACA premium tax credits expired on December 31, 2025, and KFF estimated average marketplace premium payments would rise 114% for more than 20 million enrollees. Medicaid work requirements must be in place in every state by January 1, 2027 under the 2025 reconciliation law (OBBBA).
- **Financing:** OBBBA limits new and existing state Medicaid provider taxes, with reductions beginning October 1, 2027. State supplemental payments to hospitals are exposed.
- **Opportunity:** the REH designation pays a flat monthly facility payment ($295,051.54 in CY 2026, about $3.54M a year) plus 5% above OPPS rates. The $50B Rural Health Transformation Program (RHTP) distributes $10B a year to states from 2026 through 2030. A final No Surprises Act dispute-resolution (IDR) rule issued May 28, 2026 cut dispute fees from $115 to $15 per party.

Two Medicaid shocks arrive within 12 months; RHTP funds must be spent first

Dated federal changes affecting a rural hospital, December 2025 to October 2027



Red diamonds cut revenue; green diamonds are funding or relief.

dates from CMS, HHS and law-firm summaries listed in Sources · Dec 2025 to Oct 2027

The subsidy loss is already in FY2026 results; the Medicaid changes have not yet reached the hospital's books.

The board's decision: **hold, convert to REH, or transform as a CAH, and what should trigger a change of course?**

Hospital profile (illustrative)

Attribute	Prairie Ridge Community Hospital
Type	25-bed CAH, nonprofit, Medicaid-expansion state
Service area	3 counties, about 28,000 residents
Average daily inpatient census	6.2 (annual average length of stay 3.9 days)
Inpatient admissions per year	580
ED visits per year	11,400
Outpatient (non-ED) visits per year	38,000
Staffing	212 FTEs
Payer mix (share of net patient revenue)	Medicare 48% · Medicaid 17% · Commercial 27% · Self-pay 8%

Attribute	Prairie Ridge Community Hospital
Operating revenue / expenses (FY2026)	$30.0M / $31.4M
Operating margin (FY2026)	−4.7%

2. Methodology & Regulatory Frameworks Applied

The analysis pairs a payment-policy review with a three-scenario financial model, so every recommendation traces to a specific rule and a specific dollar assumption.

Regulatory frameworks

Framework	What it requires or provides	How it shaped the analysis
Medicare CAH rules	Up to 25 inpatient beds; annual average length of stay of 96 hours or less; Medicare cost-based payment (101% of reasonable cost, before sequestration)	Defines what Prairie Ridge keeps under the status quo and loses under REH conversion
Rural Emergency Hospital designation (CAA 2021, sec. 125)	Must have been a CAH or a rural hospital with 50 or fewer beds on Dec 27, 2020; staffed ED 24/7; transfer agreement with a Level I or II trauma center; no inpatient care except a distinct-part SNF; annual average per-patient stay of 24 hours or less. Pays $295,051.54 per month (CY 2026) plus 5% over OPPS rates; patient coinsurance is figured without the 5%	Inputs and eligibility checklist for Scenario B

Framework	What it requires or provides	How it shaped the analysis
EMTALA	Screening and stabilization duties for any hospital with an emergency department, including CAHs and REHs	Sets a staffing floor in every scenario; ED cost cannot be cut to zero
Medicaid financing and eligibility (OBBBA, P.L. 119-21)	Work requirements by Jan 1, 2027 (earlier at state option); limits on state provider taxes, with reductions beginning Oct 1, 2027; CBO estimates about $911B in federal Medicaid reductions over 10 years	Drives the revenue-at-risk assumptions and the timing of decision gates
ACA marketplace subsidies	Enhanced premium tax credits expired Dec 31, 2025; reporting through April 2026 showed no extension	Drives the marketplace attrition assumption
No Surprises Act and federal IDR	Bars balance billing for emergency care. May 28, 2026 final rule: IDR fee $115 to $15 per party (disputes started on or after June 11, 2026); batching limit 25 to 50 line items; standardized payer denial codes; 3-business-day response and 5-business-day eligibility determination; new IDR Gateway portal planned	Basis for the revenue-cycle initiative on out-of-network emergency claims
Rural Health Transformation Program	$50B over five years ($10B a year, 2026 to 2030); first-year state awards range from $147M to $281M; half of funds split equally among states	Funds flow through state plans, so the hospital's lever is aligning projects with its state's plan

Confirm CAH rules against current CMS Conditions of Participation before publishing; they are stated here from general program knowledge rather than a source I opened.

Analytic methods

1. **Baseline build.** FY2026 revenue by service line and payer, expense by category, 212 FTEs.
2. **Policy-shock overlay.** Three revenue headwinds sized from payer mix and stated attrition assumptions (Section 3).
3. **Three-scenario comparison.** Hold as a CAH, convert to REH, or transform as a CAH. Each is modeled at FY2028 steady state, with the same headwinds applied to all and expenses held at FY2026 dollars for comparability.
4. **Sensitivity testing.** The three assumptions that move the answer most: the Medicare outpatient payment shift under REH, the Medicaid disenrollment rate, and how fast new programs ramp.
5. **Decision-gate framework.** Pre-agreed financial, volume and workforce triggers that move the board from Scenario C to Scenario B.
6. **Community-need check.** The community health needs assessment and HRSA's REH Health Services and Needs Assessment tool, listed in the Rural Health Information Hub resources, test whether the community can live without local inpatient beds.

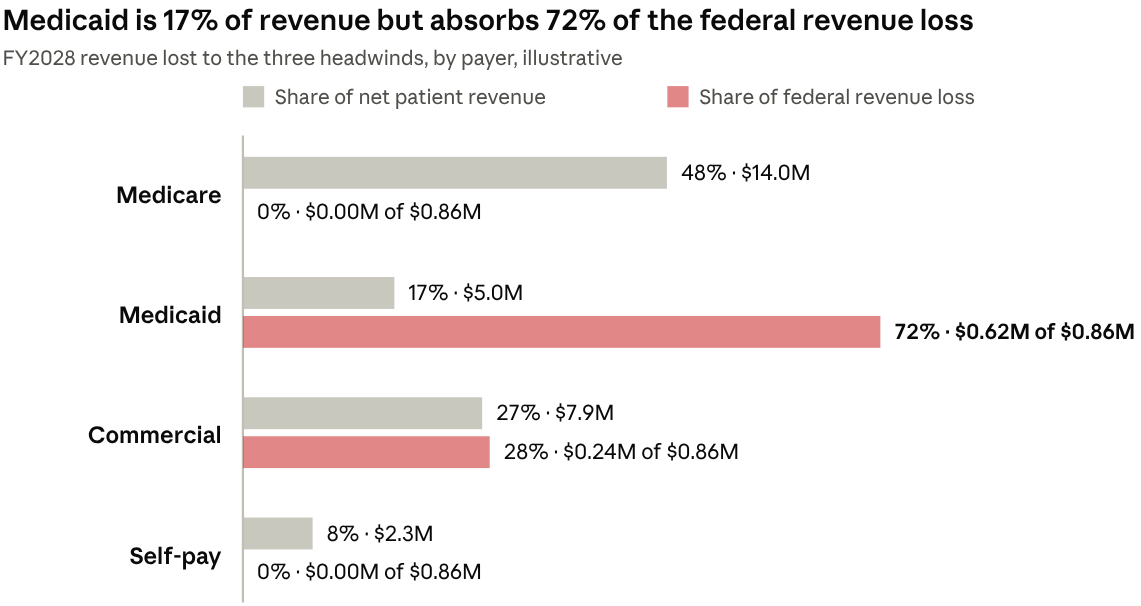
3. Mock Data & Strategic Interventions

Federal policy removes about $0.86M a year (2.9%) of Prairie Ridge's revenue by FY2028; only the transformation path closes that gap without giving up inpatient care. All figures are illustrative, in $ millions unless noted.

A. Policy headwinds applied to the baseline

Headwind	Starts	Illustrative assumption	Annual revenue impact ($M)
Medicaid work requirements	Jan 1, 2027	Expansion adults are 35% of Medicaid revenue ($5.0M); 18% lose coverage	−0.32
Marketplace subsidy expiration	Jan 1, 2026	Marketplace plans are 12% of commercial revenue ($7.9M); 25% drop coverage	−0.24
State Medicaid financing pressure	FY2028	State supplemental payments fall by 6% of Medicaid revenue	−0.30

Headwind	Starts	Illustrative assumption	Annual revenue impact ($M)
Total			**−0.86**



illustrative model · payer mix and headwind table above, $M

Medicare, the largest payer, carries none of the modeled loss under the CAH path; the exposure sits in Medicaid and, to a lesser degree, marketplace plans.

B. Scenario comparison at FY2028 steady state

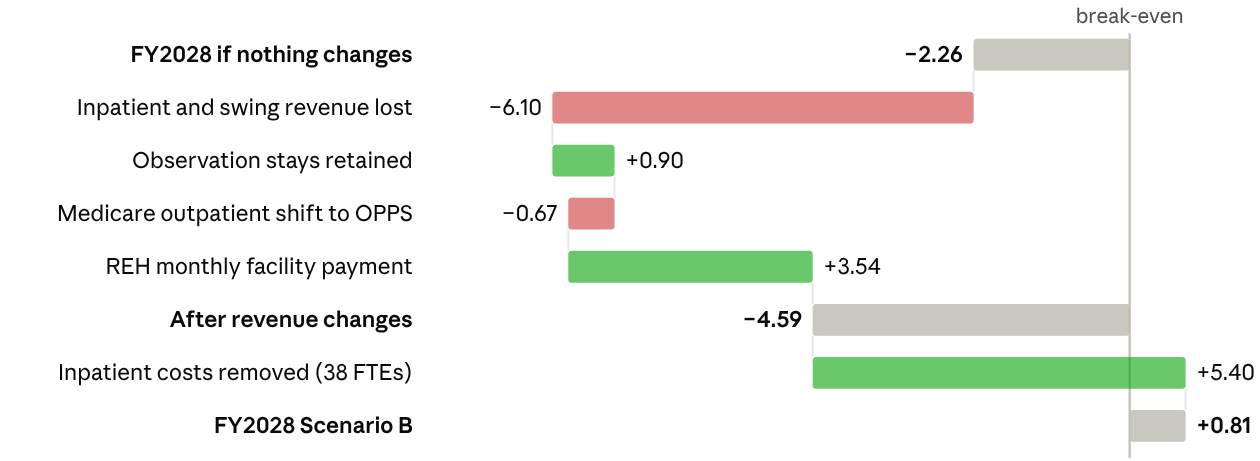
Metric	A: Hold as CAH	B: Convert to REH	C: CAH transformation (recommended)
Operating revenue ($M)	29.1	26.8	31.8
Operating expenses ($M)	31.4	26.0	31.5
Operating result ($M)	−2.3	+0.8	+0.3
Operating margin	−7.8%	+3.0%	+0.9%
FTEs	212	174	222

Metric	A: Hold as CAH	B: Convert to REH	C: CAH transformation (recommended)
Local inpatient and swing-bed care	Yes	No; about 580 admissions a year move elsewhere	Yes
Medicare payment basis	Cost-based	OPPS + 5% plus monthly facility payment	Cost-based
Reversibility	n/a	Plan as irreversible (confirm with CMS)	Fully reversible; REH option kept open

How Scenario B revenue is built, starting from $30.0M: inpatient and swing revenue −$6.1M, retained observation stays +$0.9M, Medicare outpatient shift from cost-based to OPPS + 5% −$0.67M (assumed 6% lower on $11.1M of Medicare outpatient revenue), REH facility payment +$3.54M (12 × $295,051.54), headwinds −$0.86M. Scenario B expenses fall $5.4M and 38 FTEs as inpatient staffing and ancillary costs come out.

REH conversion pays only if $5.4M of inpatient cost actually comes out

FY2028 operating result, $ millions, Scenario A to Scenario B, illustrative



Green improves the operating result, red reduces it; grey bars are totals and subtotals.

illustrative model · Scenario B build above, $M

Net revenue falls $2.33M, so the $3.54M facility payment does not replace the lost inpatient business; the result turns positive only because costs fall $5.4M.

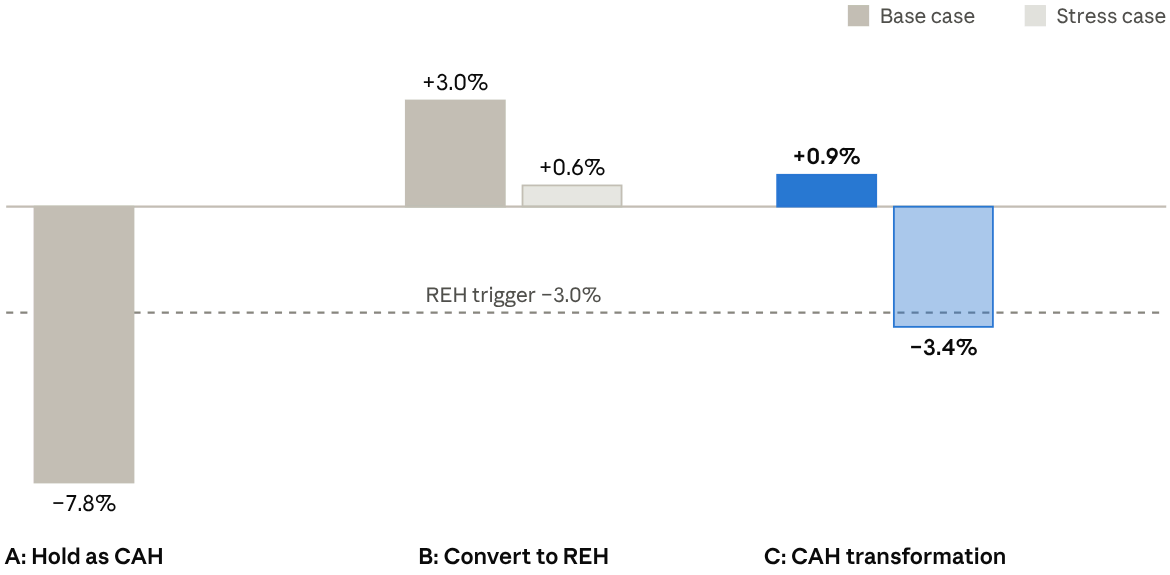
Scenario B earns the best margin, but at the price of local inpatient access and 38 jobs. Scenario C reaches near break-even and keeps the community's beds.

Sensitivity. Both answers are fragile:

- If Scenario C initiatives deliver only half their revenue uplift, FY2028 margin is −3.4%. That trips the REH trigger in Part D.
- If the Medicare outpatient shift under REH is 12% instead of 6%, Scenario B margin falls from +3.0% to +0.6%. The REH payment advantage depends on how OPPS compares with a hospital's actual costs.

Scenario C holds near break-even; a half-speed ramp breaches the REH trigger

FY2028 operating margin, illustrative



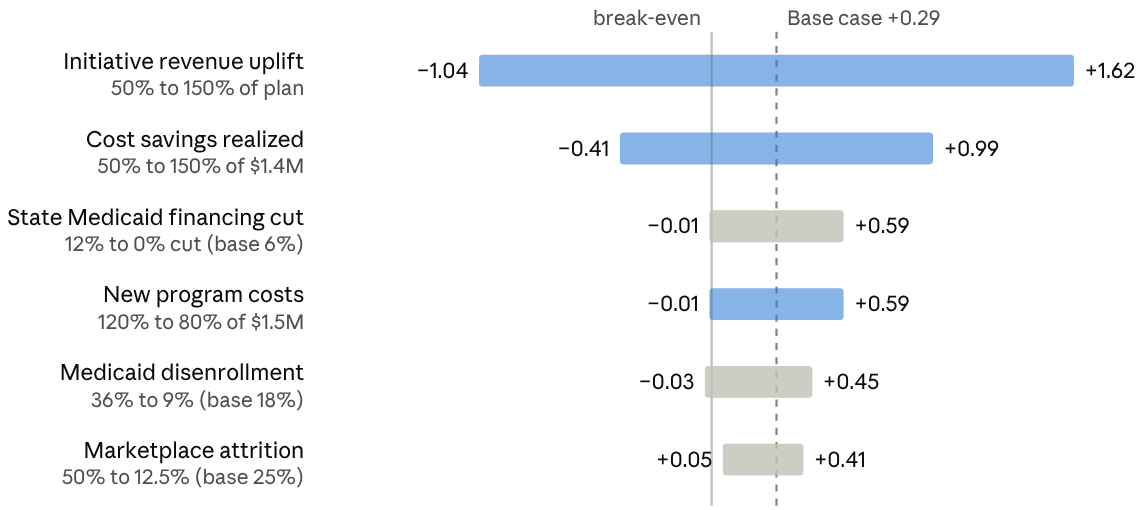
Stress case: Scenario C initiatives deliver half their revenue uplift;
Scenario B Medicare outpatient shift is 12% instead of 6%. Scenario A is shown at base only.

illustrative model · Section 3 scenario table and sensitivity cases

Only Scenario C's stress case crosses the trigger, which is why the board policy pairs the transformation plan with a pre-authorized REH study.

Execution assumptions swing Scenario C far more than federal policy assumptions

FY2028 operating result, $ millions, as each assumption moves alone, illustrative



Blue bars are execution assumptions; grey bars are federal policy assumptions. Left end is the worse case.

illustrative model · each assumption varied alone, $M

Added together, the three policy assumptions swing the result by about $1.4M, against $4.7M for the three execution assumptions, so the monthly ramp review matters more than forecasting Congress.

C. Recommended interventions (Scenario C run-rate by FY2028, $M)

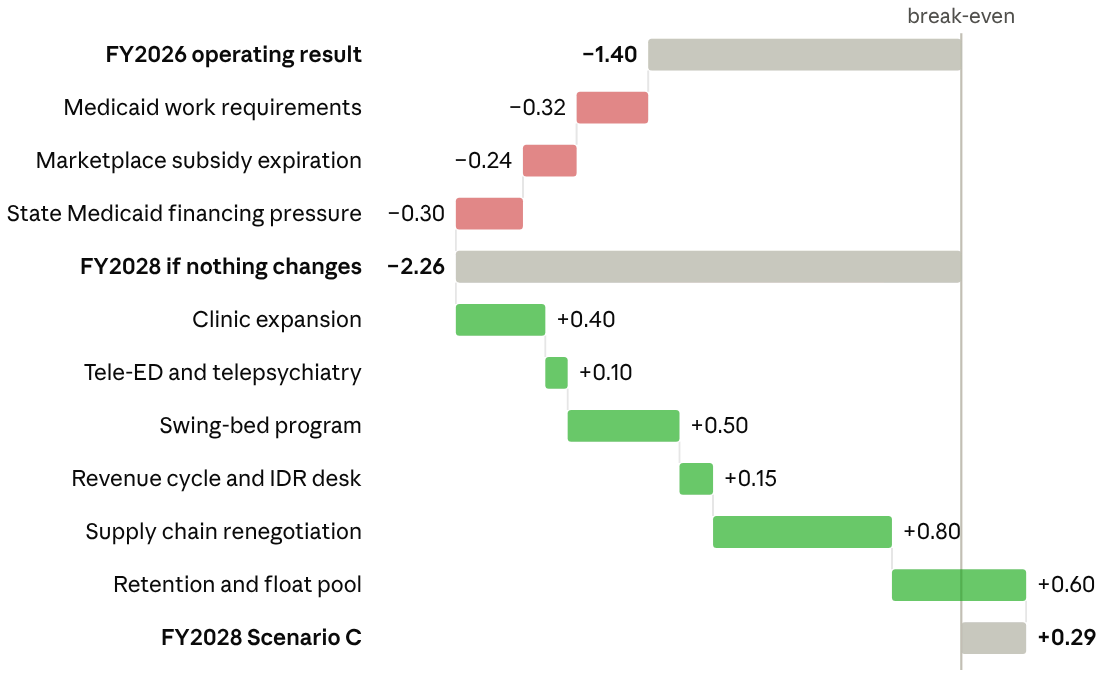
#	Initiative	Policy hook	Revenue gain or savings	New cost	Net
1	Primary care and behavioral health clinic expansion (provider-based rural health clinic)	RHTP access and workforce priorities	1.20	0.80	0.40
2	Tele-emergency and telepsychiatry; target cut in ED transfers from 6.0% to 4.5% (684 to 513 a year)	RHTP technology priorities	0.50	0.40	0.10
3	Swing-bed utilization program	CAH cost-based payment	0.60	0.10	0.50

#	Initiative	Policy hook	Revenue gain or savings	New cost	Net
4	Revenue cycle upgrade and a No Surprises Act IDR desk (about 340 eligible out-of-network emergency claims × $1,030 average recovery); A/R days 58 to 46 releases about $0.96M of one-time cash	May 2026 IDR rule: $15 fee, 50-line batching	0.35	0.20	0.15
5	Supply chain and purchased-services renegotiation	Margin protection	0.80	0.00	0.80
6	Retention program and float pool to cut agency labor	Workforce stability	0.60	0.00	0.60
	Total		**4.05**	**1.50**	**2.55**

The $2.55M net gain is the gap between Scenario C (+$0.29M) and Scenario A (−$2.26M). Separately, the plan assumes a state-administered RHTP subgrant of about $0.9M a year for three years. It funds start-up costs and is kept out of operating results because it is non-recurring.

Six initiatives turn a projected $2.3M loss into a $0.3M gain by FY2028

Operating result, $ millions, illustrative



Green improves the operating result, red reduces it; grey bars are totals.

illustrative model · Section 3 headwind and initiative tables, $M

Supply-chain and agency-labor savings are within management's control and supply $1.40M of the $2.55M; the rest depends on program ramp and the state subgrant.

D. Mock board policy outline: Rural Access & Financial Resilience Policy

1. **Purpose.** Preserve local emergency and inpatient access while the hospital's finances absorb federal coverage and financing changes.
2. **Standing strategy.** Operate as a CAH and execute initiatives 1 to 6 in Part C, with quarterly reporting by the CFO and COO.
3. **REH review triggers.** Any two of the following start a 90-day REH feasibility study:
   - Trailing 12-month operating margin below −3.0% for two consecutive quarters
   - Average daily census below 4.0 for two consecutive quarters (baseline 6.2)
   - Medicaid expansion enrollment in the service area down more than 25% from December 2026
   - Unfilled RN or provider positions above 20% for two quarters
   - No state RHTP subgrant secured by the end of Q2 FY2027
4. **Conversion decision.** The board votes on conversion after the study, with the community health needs assessment, a transfer-agreement plan with a Level I or II trauma center, and a workforce transition plan in hand.
5. **Compliance guardrails.** EMTALA screening and stabilization in every scenario; no balance billing for covered emergency care under the No Surprises Act; good-faith estimates for self-pay patients; IDR filings tracked against statutory deadlines.

4. Key Deliverables & Outcomes

Executed as planned, the transformation path turns a projected $2.3M operating loss into a $0.3M gain by FY2028, a $2.55M swing, without closing a single inpatient bed. Targets below are illustrative projections, not results.

Projected outcomes

KPI	FY2026 baseline	FY2028 if nothing changes	FY2028 target (Scenario C)
Operating result ($M)	−1.4	−2.3	+0.3
Operating margin	−4.7%	−7.8%	+0.9%

KPI	FY2026 baseline	FY2028 if nothing changes	FY2028 target (Scenario C)
ED transfer rate	6.0% (684 a year)	6.0%	4.5% (513 a year)
Days in accounts receivable	58	58	46
Out-of-network emergency claim recoveries ($M a year)	n/a	n/a	+0.35
FTEs	212	212	222

Deliverables in the case-study package

Deliverable	Audience	Purpose
Board decision memo (4 pages)	Board, CEO	Recommend the path and the REH triggers
Scenario financial model (workbook)	CFO	Scenarios A to C, headwind inputs, sensitivities
Policy impact brief (2 pages)	Executive team	Plain-language effects of OBBBA, subsidy expiration, REH, NSA and RHTP
KPI dashboard	COO, CFO	Quarterly tracking of the five trigger metrics
Implementation roadmap	All leaders	24-month plan with decision gates
Board policy	Board	Rural Access & Financial Resilience Policy

Risks and mitigations

Risk	Early-warning signal	Mitigation
New programs ramp slower than modeled	Clinic and tele-ED volume under 60% of plan after two quarters	Monthly ramp review; the −3.0% margin trigger starts the REH study

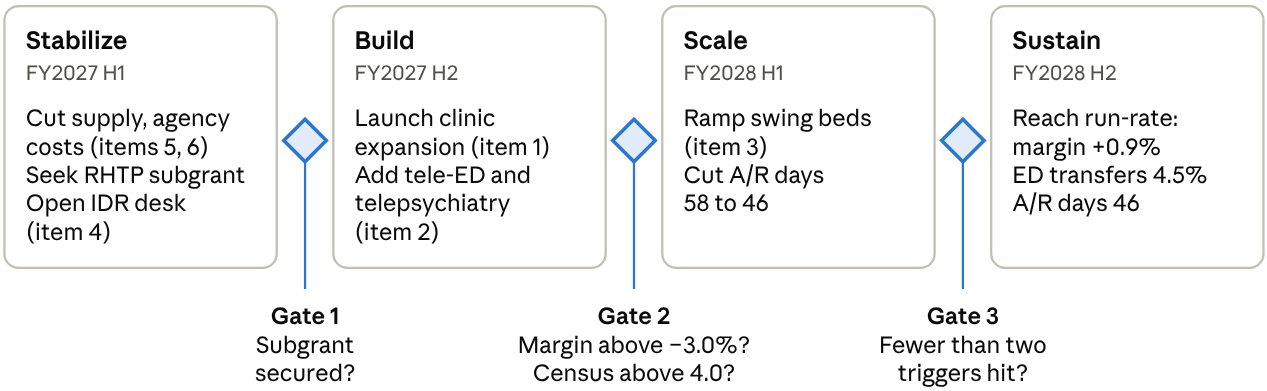
Risk	Early-warning signal	Mitigation
State plan does not fund the hospital	No subgrant by end of Q2 FY2027	Align each initiative to the state's RHTP priorities; pursue regional partner applications
Coverage losses exceed assumptions	Expansion enrollment down more than 25% from December 2026	Enrollment-assistance outreach; revisit self-pay and charity care policy
Federal rules shift again (subsidies, work-requirement timing, IDR litigation)	New rule, court order or extension	Re-run the model; headwinds are inputs, not hard-coded
Workforce shortage forces service cuts	Vacancy rate above 20%	Retention bonuses, float pool, tele-specialty coverage
IDR filings missed or rejected	Rejection rate above 10%	Standard operating procedure built on the May 2026 rule; payer lookup in the IDR registry

What this project demonstrates

- **Policy fluency:** translating Medicare, Medicaid, ACA and No Surprises Act changes into dollar effects for one facility.
- **Financial judgment:** building comparable scenarios, stating assumptions, and testing which ones break the answer.
- **Executive framing:** a recommendation with decision triggers, not a single bet.
- **Compliance awareness:** EMTALA, patient billing protections and CMS designation rules built into the plan.

Implementation roadmap

Three gates in 24 months decide whether Prairie Ridge stays a CAH



Federal dates: Medicaid work requirements start Jan 1, 2027; state provider-tax reductions begin Oct 1, 2027.

Two or more triggers at any gate start a 90-day REH feasibility study.

implementation roadmap · 4 phases, 3 gates

Savings and the funding application come first, new programs follow, and each gate checks the same triggers the board policy lists.

5. How AI Addresses the Problem

AI does not change the payment rules; it cuts the cost of running the six initiatives and gives the board an early warning on the REH triggers. In this illustrative model, $0.30M a year of AI tooling sits inside the existing $1.5M new-cost line and touches initiatives that carry $1.45M of the $4.05M gross gain.

AI is a supporting tool here, not the solution. A review cited by KFF Health News found only 26 peer-reviewed studies of AI in rural health care from 2010 through April 2025, and many state RHTP plans track adoption rather than outcomes. One hospital technology leader quoted there said AI cannot fix closures driven by financial pressure. The plan therefore treats every AI claim as a hypothesis with a measured target.

Where AI fits

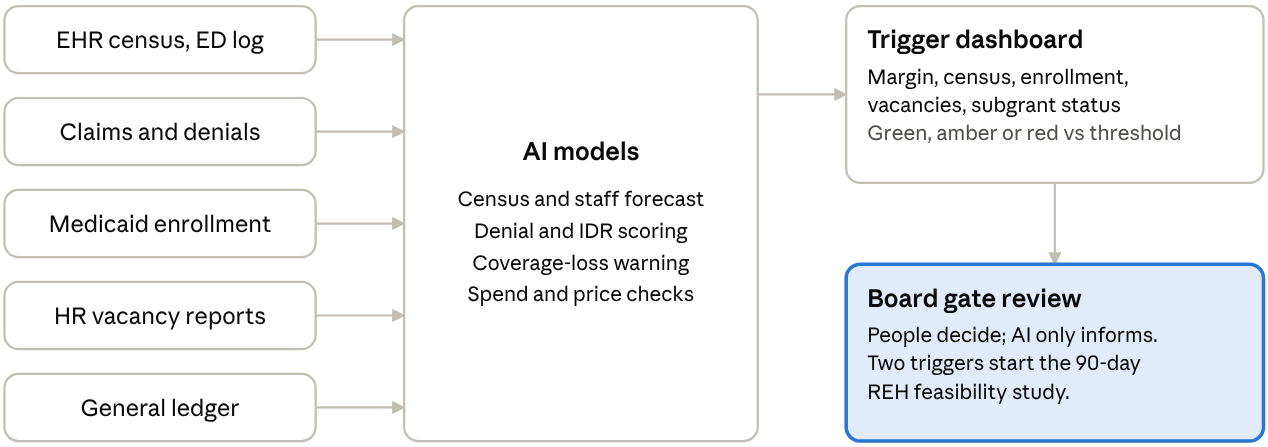
AI use	Problem it addresses	Linked to	Illustrative target	Main guardrail
Revenue-cycle AI: coding review, denial prediction, claim-status checks	Slow cash and avoidable denials	Initiative 4	A/R days 58 to 46, releasing about $0.96M of one-time cash	Human coder reviews low-confidence claims; monthly audit sample
IDR claim triage	Out-of-network emergency claims go unfiled. The May 2026 rule requires an eligibility attestation with supporting documents when a dispute starts and a response within 3 business days	Initiative 4	About 340 eligible claims × $1,030 = $0.35M a year	A compliance lead signs every filing; deadlines tracked
Tele-ED decision support: triage, sepsis and stroke alerts	Unneeded transfers and late recognition	Initiative 2	ED transfers 6.0% to 4.5% (171 fewer a year)	Clinician decides; alert thresholds tested on local data
Ambient documentation	Charting burden, burnout and agency-labor reliance	Initiative 6	Supports the $0.6M agency-labor savings; track charting hours and vacancy rate	Patient notice; clinician reviews every note

AI use	Problem it addresses	Linked to	Illustrative target	Main guardrail
Staffing and census forecasts	Float pool sizing and agency spend	Initiative 6, vacancy trigger	Weekly forecast error tracked against actual census	Forecast informs, manager decides
Spend analytics	Overpaying suppliers and contracts	Initiative 5	Supports the $0.8M savings target	Savings verified in the general ledger, not the tool
Coverage-loss early warning	Patients losing Medicaid once work requirements start	Medicaid enrollment trigger	If outreach cuts expansion disenrollment from 18% to 14%, about $0.07M a year (outside the base plan)	Outreach limited to existing patients; privacy review
Trigger early-warning dashboard	Board learns of a breach only after two bad quarters	All five REH triggers	Monthly view instead of quarterly	People decide; AI never triggers a conversion

Tooling budget (assumption): $0.10M inside the revenue-cycle line, $0.10M inside the tele-ED line and $0.10M for ambient documentation inside the clinic line, $0.30M in all. Start with revenue-cycle and IDR triage in the Stabilize phase: $0.10M of tooling against $0.35M a year in recoveries plus the one-time cash release.

Early-warning flow

AI turns five data feeds into trigger warnings; the board still makes the call



early-warning flow · 5 feeds, 1 model layer, 1 human gate

The models watch the same five metrics the board policy lists, monthly, and only the board decides whether a gate is passed.

What can go wrong

- **Privacy.** Any tool that touches patient data needs a HIPAA business associate agreement and a data-use review before a pilot.
- **Accuracy.** AI notes, codes and alerts can be wrong, so a person reviews anything before it reaches the chart or a payer.
- **Rural fit.** The KFF Health News reporting notes tools are often trained on urban data and many rural sites lack IT staff and fast internet. Check connectivity and training capacity first.
- **Payers use AI too.** CMS's WISeR model applies AI-assisted prior authorization to selected Traditional Medicare services in six states (Arizona, New Jersey, Ohio, Oklahoma, Texas, Washington). Its future was contested in Congress as of mid-2026, so check its current status if your state is covered.
- **Unmeasured claims.** CMS has no AI-specific reporting requirement for RHTP states, so set hours-saved, denial-rate and recovery targets for each tool before launch.
- **Community trust.** Residents interviewed by KFF Health News were wary of AI in personal care; disclose its use and keep a clinician in every clinical decision.

AI artifacts to add to your portfolio

- A one-page AI use-case map (the table above) with your own targets and guardrails.
- A one-page AI governance policy: approval steps, privacy review, human sign-off, outcome reporting.
- A small prototype using synthetic data: a census forecast and trigger-status tracker in Excel or Python.
- An ROI tab in the scenario workbook: tooling cost against benefit, with low, base and high cases.

The HIPAA and clinical-accuracy points are general practice, not drawn from a source I opened; confirm them with your compliance team.

6. Visual Assets & Artifacts to Build

Build six assets in this order: together they let a recruiter grasp the project in 60 seconds and test your depth in five minutes.

#	Asset	Format	What it must show
1	Executive slide deck (10 slides)	PowerPoint or PDF	Slide flow: problem and four policy shifts; hospital profile; headwinds; scenario comparison; recommendation; six initiatives; roadmap with gates; risks; board ask. One message per slide title
2	Scenario financial model	Excel workbook	Tabs: Inputs, Baseline, Headwinds, Scenarios A to C, Sensitivity. Blue cells for inputs, black for formulas, every assumption labeled; a reader should be able to change the Medicaid disenrollment rate and watch the margin move

#	Asset	Format	What it must show
3	KPI dashboard mockup	Tableau, Power BI, Excel or Figma	Operating margin, ED transfer rate, A/R days, average daily census, Medicaid enrollment, vacancy rate; each against its trigger threshold with a green, amber or red status
4	Policy impact brief (2 pages)	PDF	A table of OBBBA, enhanced ACA subsidy expiration, REH, IDR rule and RHTP: date, who is affected, effect on a rural hospital, recommended action
5	Board decision memo and policy (4 pages)	Word or PDF	Recommendation first, then options, financials, triggers and the draft Rural Access & Financial Resilience Policy from Section 3
6	One-page portfolio summary	Web page or PDF	The hero visual (a bar chart of FY2028 operating margin: −7.8%, +3.0%, +0.9%), three headline numbers, your role, tools used, links to assets 1 to 5

Optional extras if you have time: a stakeholder map (power against interest: board, CEO, medical staff, state Medicaid office, county commissioners, payers, patients), and a one-page patient communication sheet explaining No Surprises Act protections in plain language.

How to present it

- **Lead with the decision, not the topic.** Your portfolio headline: "Should a 25-bed rural hospital convert to an REH? A three-scenario analysis."
- **Label the case as illustrative** on every asset, and keep the real policy facts (dates, payment amounts) visibly sourced.
- **Show your model.** Recruiters in finance and operations trust a case more when they can open the assumptions.

- **Tailor the emphasis.** For hospital operations roles, lead with assets 2, 3 and 5; for policy and government-affairs roles, lead with 4 and the Section 2 framework table.
- **Reuse the template.** The same structure works for a second project on surprise billing, Medicaid financing or a medical device topic such as Medicare coverage pathways and hospital value-analysis decisions.

Before you publish

Re-check three items that were still moving in the sources I reviewed: whether Congress has extended the enhanced premium tax credits, whether any state has delayed or changed its Medicaid work-requirement start, and the final IDR Gateway launch date. Confirm the CAH rules in Section 2 against current CMS Conditions of Participation.

Sources

Opened in full:

- CMS MLN Fact Sheet: Rural Emergency Hospitals (Dec 2025)
- Jones Day: New Rule Amends No Surprises Act Dispute Process (Jun 8, 2026)
- KFF Health News: Patients Wary of Governments, Companies Pushing AI as a Rural Healthcare Solution (Aug 11, 2026)
- Georgetown CHIR: CMS's WISeR Model Faces Potential Repeal Following GAO Determination

Read as search excerpts only; verify before you publish:

- CMS press release: $50 billion in Rural Health Transformation awards (Dec 29, 2025)
- Becker's: CMS awards $50B to states for rural health transformation
- TruBridge: What OBBBA means for hospitals, key dates
- PracticeLink: OBBBA healthcare overview (CBO Medicaid estimate)
- Advisory Board daily briefing, Jan 7, 2026 (KFF premium analysis)
- ASTHO: ACA enhanced premium tax credits, legislative developments (Jan 20, 2026)
- Minnesota Hospital Association: Rural Emergency Hospitals (CY 2026 payment)
- Rural Health Information Hub: REH resources

Board Decision Memo: Prairie Ridge Community Hospital

Oct 25, 2026 · @Neha

Recommendation: keep Prairie Ridge a Critical Access Hospital, approve the six-initiative transformation plan, and adopt the REH trigger policy. In this illustrative model, that path moves FY2028 operating margin from −7.8% under the status quo to +0.9% while keeping inpatient care in the community.

Decision requested

The board is asked to take three actions at its next meeting:

1. **Approve the six-initiative transformation plan.** It returns a net $2.55M a year by FY2028 against $1.50M of new annual cost.
2. **Adopt the Rural Access & Financial Resilience Policy.** Five triggers are tracked monthly; any two start a 90-day Rural Emergency Hospital (REH) feasibility study.
3. **Direct management to pursue a state Rural Health Transformation Program (RHTP) subgrant** of about $0.9M a year for three years (an assumption) and to report at three decision gates.

Situation: why now

Prairie Ridge already loses $1.4M a year (−4.7% margin on $30.0M of revenue), and federal changes add $0.86M of revenue loss by FY2028, pushing the loss to $2.26M (−7.8%) if nothing changes. Medicaid is 17% of revenue but absorbs 72% of that new loss.

Date	Federal change	Effect on Prairie Ridge
Jan 1, 2026	Enhanced ACA premium tax credits expired	Marketplace attrition: −$0.24M a year
Jun 11, 2026	No Surprises Act dispute fee falls from $115 to $15 per party; batching limit rises from 25 to 50 line items	Lower cost to pursue out-of-network emergency claims

Date	Federal change	Effect on Prairie Ridge
Jan 1, 2027	Medicaid work requirements due in every state	Expansion-adult coverage loss: −$0.32M a year
Sep 30, 2027	Spend-by date for FY2026 RHTP funds (per CMS guidance, as reported by state hospital associations)	Deadline for any subgrant start-up money
Oct 1, 2027	State Medicaid provider-tax reductions begin	State supplemental payment pressure: −$0.30M a year

The REH designation offers a flat monthly facility payment of $295,051.54 in 2026 (about $3.54M a year) plus 5% over OPPS rates, but only for hospitals that give up inpatient care.

Options compared

Converting to an REH earns the best margin (+3.0%) but ends local inpatient care and 38 jobs; the transformation plan reaches +0.9% and keeps both. FY2028 steady state, illustrative:

Metric	A: Hold as CAH	B: Convert to REH	C: CAH transformation (recommended)
Operating revenue ($M)	29.1	26.8	31.8
Operating expenses ($M)	31.4	26.0	31.5
Operating result ($M)	−2.3	+0.8	+0.3
Operating margin	−7.8%	+3.0%	+0.9%
FTEs	212	174	222
Local inpatient and swing-bed care	Yes	No; about 580 admissions a year move elsewhere	Yes
Medicare payment basis	Cost-based	OPPS + 5% plus monthly facility payment	Cost-based

Metric	A: Hold as CAH	B: Convert to REH	C: CAH transformation (recommended)
Reversibility	n/a	Plan as irreversible (confirm with CMS)	Fully reversible

Both favorable results are fragile. If Scenario C initiatives deliver half their revenue uplift, FY2028 margin is −3.4%. If the Medicare outpatient payment shift under REH is 12% instead of 6%, Scenario B falls to +0.6%. REH conversion turns positive only because $5.4M of inpatient cost comes out, against a $2.33M net revenue loss.

Recommended plan and budget

Six initiatives return $4.05M of revenue and savings for $1.50M of new cost, a net $2.55M a year by FY2028; $1.40M of it is cost action within management's control. Run-rate figures, $M:

#	Initiative	Gain or savings	New cost	Net
1	Primary care and behavioral health clinic expansion	1.20	0.80	0.40
2	Tele-emergency and telepsychiatry (ED transfers 6.0% to 4.5%)	0.50	0.40	0.10
3	Swing-bed utilization program	0.60	0.10	0.50
4	Revenue cycle upgrade and No Surprises Act dispute desk	0.35	0.20	0.15
5	Supply chain and purchased-services renegotiation	0.80	0.00	0.80
6	Retention program and float pool to cut agency labor	0.60	0.00	0.60
	Total	**4.05**	**1.50**	**2.55**

AI tooling of about $0.30M a year sits inside the new-cost lines ($0.10M each in revenue cycle, tele-ED and clinic documentation). Cutting accounts-receivable days from 58 to 46 also releases about $0.96M of one-time cash. A state RHTP subgrant of about $0.9M a

year for three years, if awarded, funds start-up costs and is kept out of operating results.

Triggers and decision gates

Any two of five triggers start a 90-day REH feasibility study, so conversion is a pre-agreed response rather than a crisis decision. Management reports all five monthly.

- Trailing 12-month operating margin below −3.0% for two consecutive quarters
- Average daily census below 4.0 for two consecutive quarters (baseline 6.2)
- Medicaid expansion enrollment in the service area down more than 25% from December 2026
- Unfilled RN or provider positions above 20% for two quarters
- No state RHTP subgrant secured by the end of Q2 FY2027

Three gates review them: **Gate 1** (end of FY2027 H1) asks whether the subgrant is secured; **Gate 2** (end of FY2027 H2) asks whether margin is above −3.0% and census above 4.0; **Gate 3** (end of FY2028 H1) asks whether fewer than two triggers are hit. If the study is triggered, the board votes on conversion with the community health needs assessment, a transfer agreement with a Level I or II trauma center, and a workforce transition plan in hand. EMTALA screening and stabilization duties apply in every scenario.

Risks and the first 90 days

The plan's largest risk is execution, not policy: the three execution assumptions swing the result by about $4.7M added together, against about $1.4M for the three federal policy assumptions.

Risk	Early-warning signal	Mitigation
New programs ramp slower than modeled	Clinic and tele-ED volume under 60% of plan after two quarters	Monthly ramp review; the margin trigger starts the REH study
State plan does not fund the hospital	No subgrant by end of Q2 FY2027	Align each initiative to the state's RHTP priorities; pursue regional partner applications

Risk	Early-warning signal	Mitigation
Coverage losses exceed assumptions	Expansion enrollment down more than 25%	Enrollment-assistance outreach; revisit charity care policy
Workforce shortage forces service cuts	Vacancy rate above 20%	Retention bonuses, float pool, tele-specialty coverage
AI tools underperform or mishandle data	Error rate above target; privacy review findings	Business associate agreement, human review of every output, targets set before launch

First 90 days

- [ ] Launch the supply chain and purchased-services review (initiative 5)
- [ ] Stand up the dispute desk and revenue-cycle AI pilot (initiative 4)
- [ ] Submit the state RHTP subgrant application and meet the state Medicaid office
- [ ] Start monthly reporting of the five trigger metrics
- [ ] Open recruitment for the retention program and float pool (initiative 6)
- [ ] Schedule Gate 1 for the end of FY2027 H1

Prairie Ridge is a fictional hospital and all hospital figures are illustrative assumptions. Policy facts are sourced in the full case study.

BOARD DECISION BRIEFING · OCTOBER 2026

Keeping rural care open at Prairie Ridge

A Critical Access Hospital transformation plan against the 2026 to 2028 policy squeeze

Illustrative case study. Prairie Ridge Community Hospital is fictional; all hospital figures are modeled assumptions.

THE PROBLEM

A 25-bed hospital is losing money before the policy changes land

−4.7%
FY2026 operating margin

−$1.4M
Operating result on $30.0M revenue

6.2
Average daily inpatient census

48%
Of patient revenue from Medicare

Medicaid is 17% of patient revenue, commercial 27% and self-pay 8%. The hospital has 212 FTEs, 11,400 ED visits and 580 admissions a year.

HEADWINDS

Three policy changes remove $0.86M a year by FY2028

STARTS JAN 1, 2027

Medicaid work requirements

−$0.32M

18% of expansion adults lose coverage, on 35% of Medicaid revenue.

CREDITS EXPIRED DEC 31, 2025

Marketplace subsidies

−$0.24M

25% of marketplace enrollees drop coverage, on 12% of commercial revenue.

FROM OCT 1, 2027

State Medicaid financing

−$0.30M

Provider-tax reductions cut state supplemental payments by 6% of Medicaid revenue.

Medicaid absorbs 72% of the loss. Holding course widens the deficit from −$1.4M to −$2.26M.

OPTIONS

Only conversion and transformation clear the loss

FY2028 scenario	Result	Margin	FTEs	Care model
A: Hold as CAH	−$2.26M	−7.8%	212	Inpatient and ED kept
B: Convert to REH	+$0.81M	+3.0%	174	Inpatient ends; ED and outpatient kept
C: CAH transformation	+$0.29M	+0.9%	222	Inpatient and ED kept; services added

The REH facility payment is $295,051.54 a month for CY2026, about $3.54M a year, but it replaces cost-based inpatient revenue and shifts Medicare outpatient pay to OPPS plus 5%.

RECOMMENDATION

Transform as a Critical Access Hospital

+$0.29M
FY2028 operating result, +0.9% margin

+$2.55M
Better than holding as a CAH

222
FTEs, ten more than today

REH has the higher margin on paper but is fragile. A 12% Medicare outpatient shift cuts it to +0.6%, and it ends inpatient care and 38 positions. REH stays the fallback, set off by board triggers.

THE PLAN

Six initiatives add $2.55M a year, net

Initiative	Gain ($M)	New cost ($M)	Net ($M)
1. Clinic expansion	1.20	0.80	0.40
2. Tele-ED and telepsychiatry	0.50	0.40	0.10
3. Swing-bed program	0.60	0.10	0.50
4. Revenue cycle and dispute desk	0.35	0.20	0.15
5. Supply chain renegotiation	0.80	0.00	0.80
6. Retention and float pool	0.60	0.00	0.60

Total: $4.05M gain, $1.50M new cost, $2.55M net. The dispute desk uses the May 2026 federal rule: $15 fee per party and batches of up to 50 line items.

ROADMAP

Four phases, three board gates

FY2027 H1

Stabilize

Supply chain, retention, subgrant, dispute desk

GATE 1

Is the subgrant secured?

FY2027 H2

Build

Clinic expansion, tele-ED and telepsychiatry

GATE 2

Is margin above −3.0% and census above 4.0?

FY2028 H1

Scale

Swing-bed program, A/R days from 58 to 46

GATE 3

Are fewer than two REH triggers tripped?

FY2028 H2

Sustain

Hold the gains and review triggers each year

TARGET

+0.9% margin, 4.5% ED transfers, 46 A/R days

Any two triggers for the stated period start a 90-day REH feasibility study: margin below −3.0%, census below 4.0, Medicaid enrollment down 25%, vacancies above 20%, no subgrant by Q2 FY2027.

AI SUPPORT

AI supports three initiatives for $300K a year

Revenue-cycle and dispute-claim AI

$175K

Base benefit on $100K of tooling. Speeds claims scrubbing and out-of-network dispute filing.

Tele-ED decision support

$125K

Base benefit on $100K of tooling. Helps keep ED transfers moving from 6.0% toward 4.5%.

Ambient clinic documentation

$120K

Base benefit on $100K of tooling. Frees clinician time in the expanded clinics.

Base return is 1.4 times cost, and the tools break even at 71% of assumed benefit. The evidence is thin: a 2026 KFF Health News review found only 26 peer-reviewed studies of AI in rural care, so the plan tracks outcomes, not adoption.

RISKS

Execution risk outweighs policy risk

Assumption moved alone	Worse case	Better case
Initiative revenue (50% to 150%)	−$1.04M	+$1.62M
Cost savings (50% to 150%)	−$0.41M	+$0.99M
State financing cut (12% to 0%)	−$0.01M	+$0.59M
Program cost (120% to 80%)	−$0.01M	+$0.59M
Medicaid disenrollment (36% to 9%)	−$0.03M	+$0.45M
Marketplace attrition (50% to 12.5%)	+$0.05M	+$0.41M

$4.7M

Result swing from execution assumptions

$1.4M

Result swing from federal policy assumptions

Base result is +$0.29M. If initiatives deliver half their revenue, margin falls to −3.4%, below the −3.0% trigger. Gate reviews catch that early.

DECISIONS REQUESTED

Three decisions for the board

1. Approve Scenario C and the $1.50M annual program budget.
2. Adopt the five REH triggers and the three decision gates.
3. Authorize the Rural Health Transformation subgrant application and the first 90-day plan.

Sources: CMS MLN Fact Sheet MLN2259384 (REH payment); Jones Day, June 2026 (No Surprises Act dispute rule); KFF Health News, August 2026. Hospital figures are illustrative.

Prairie Ridge Community Hospital: Rural Viability Dashboard

Transforming as a Critical Access Hospital moves the FY2028 operating margin from **−7.8%** if the hospital holds course to **+0.9%**, a gain of **$2.55M** a year. Converting to a Rural Emergency Hospital reaches **+3.0%** but ends inpatient care. Fictional hospital, illustrative model, $ millions.

FY2026 baseline
−4.7%
−$1.40M · 212 FTEs

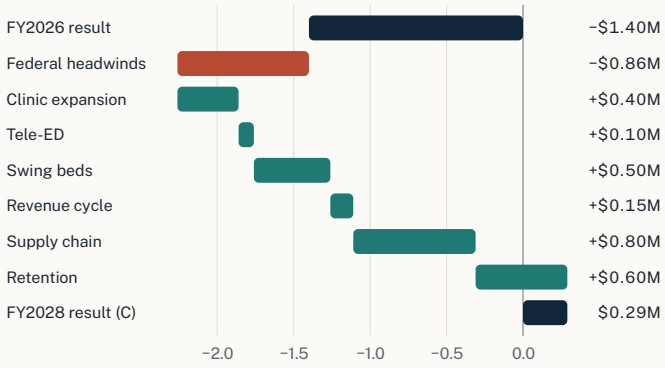
A: Hold as CAH
−7.8%
−$2.26M · 212 FTEs

B: Convert to REH
+3.0%
$0.81M · 174 FTEs

C: CAH transformation
+0.9%
$0.29M · 222 FTEs · recommended

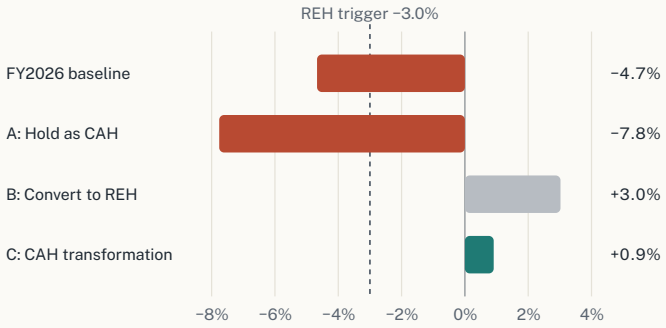
From the FY2026 loss to the Scenario C result

Annual operating result, $M. Federal headwinds hit first, then six initiatives at run-rate by FY2028.



Operating margin by scenario

FY2028 steady state. The dashed line is the board's REH review trigger.



Six initiatives in Scenario C

Run-rate by FY2028, $M a year. Net is gain or savings minus new cost.

Initiative	Type	Gain	Cost	Net
Primary care and behavioral health clinic expansion	Revenue	$1.20M	$0.80M	$0.40M
Tele-emergency and telepsychiatry	Revenue	$0.50M	$0.40M	$0.10M
Swing-bed utilization program	Revenue	$0.60M	$0.10M	$0.50M
Revenue cycle upgrade and No Surprises Act dispute desk	Revenue	$0.35M	$0.20M	$0.15M
Supply chain and purchased-services renegotiation	Savings	$0.80M	—	$0.80M
Retention program and float pool to cut agency labor	Savings	$0.60M	—	$0.60M

Total gain **$4.05M** · New cost **$1.50M** · Net **$2.55M**

Federal headwinds, annual by FY2028

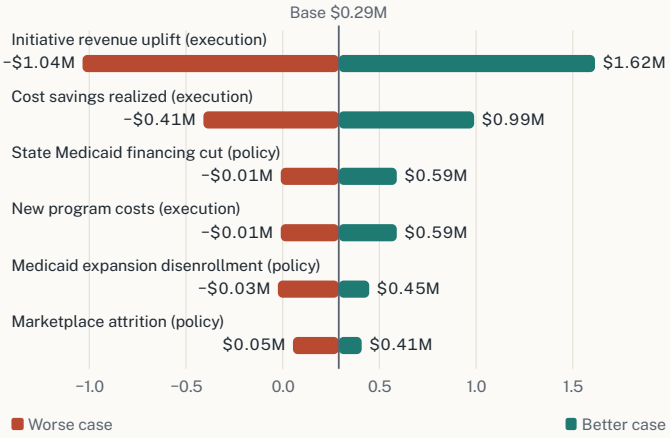
Revenue lost from three policy changes, $M. Medicaid carries most of it.

Headwind	Exposed	Rate	Impact	Starts
Medicaid work requirements	$1.75M	18%	−$0.32M	Jan 1, 2027
Marketplace subsidy expiration	$0.95M	25%	−$0.24M	Jan 1, 2026
State Medicaid financing pressure	$5.00M	6%	−$0.30M	FY2028

Loss by payer	Revenue lost	Share
Medicare	—	—
Medicaid	$0.62M	72%
Commercial	$0.24M	28%
Self-pay	—	—

What moves the Scenario C result

Annual operating result, $M, when each assumption moves alone. Execution risk outweighs policy risk.



REH triggers

Any two tripped for the stated period start a 90-day REH feasibility study. Readings are the FY2026 baseline; the rest are not yet measured.

Trigger	Threshold	Reading	Status
Trailing 12-month operating margin	Below −3%	−4.7%	Tripped
Average daily census	Below 4.0	6.2	Clear
Medicaid expansion enrollment vs Dec 2026	Down more than 25%	—	Monitor
Vacancy rate	Above 20%	—	Monitor
Rural Health Transformation subgrant	Not secured by Q2 FY2027	—	Monitor

Roadmap and decision gates

The board reviews at each gate before releasing the next phase of spending.

Gate	When	Phase	Question
Gate 1	End of FY2027 H1	Stabilize	Is the Rural Health Transformation subgrant secured?
Gate 2	End of FY2027 H2	Build	Is margin above -3.0% and census above 4.0?
Gate 3	End of FY2028 H1	Scale	Are fewer than two REH triggers tripped?
Target	End of FY2028 H2	Sustain	Run-rate margin +0.9%, ED transfers 4.5%, A/R days 46

AI tooling: cost against benefit

$M a year. Benefit is the assumed share of the linked initiative's gain; low and high are 50% and 150% of base.

Tool	Cost	Low	Base	High
Revenue-cycle and dispute-claim AI	$0.100M	$0.087M	$0.175M	$0.263M
Tele-ED decision support	$0.100M	$0.063M	$0.125M	$0.188M
Ambient clinic documentation	$0.100M	$0.060M	$0.120M	$0.180M

Total cost **$0.300M** Base benefit **$0.420M** Base return **1.4x**

Prairie Ridge is a fictional 25-bed hospital and every figure is an illustrative assumption from the companion scenario model. The REH monthly facility payment ($295,051.54 for CY2026) comes from the CMS fact sheet MLN2259384. Policy dates reflect the law and rules as of October 2026.

Prairie Ridge Community Hospital: Scenario Model

Illustrative case study model. Prairie Ridge is a fictional hospital; all hospital figures are assumptions. $ millions unless noted.

Purpose	Compares three FY2028 strategies for a 25-bed rural hospital: A) hold as a Critical Access Hospital, B) convert to a Rural Emergency Hospital (REH), C) transform as a CAH (recommended).
How to use	Change blue cells on the Inputs sheet (and the blue cells on Initiatives, Sensitivity and AI_ROI). Every other number recalculates.
Colour legend	Blue text = hardcoded input. Black = formula. Green = link to another sheet. Yellow fill = key assumption to test first.
Inputs	Every assumption in one place, with its source or a note saying it is an illustrative assumption.
Baseline	FY2026 revenue, expenses, margin and payer mix.
Headwinds	Revenue lost to the Medicaid work requirements, ACA subsidy expiration and state Medicaid financing pressure, plus the loss by payer.
Initiatives	The six transformation initiatives with gains, new costs, AI tooling and operating tie-outs.
Scenarios	Side-by-side FY2028 revenue, expenses, result, margin and FTEs, with a tie-out to the case study.
Sensitivity	Tornado table (each assumption moved alone) and stress cases against the -3.0% REH trigger.
AI_ROI	Tooling cost against attributable benefit for three AI tools, with low, base and high cases and a break-even point.
Sources	REH monthly facility payment: CMS MLN Fact Sheet MLN2259384 (Dec 2025), https://www.cms.gov/files/document/mln2259384-rural-emergency-hospitals.pdf. Dispute-fee and batching changes: Jones Day, 8 Jun 2026, https://www.jonesday.com/en/insights/2026/06/new-rule-amends-no-surprises-act-dispute-process-for-outofnetwork-health-care-payments. Everything else is an illustrative assumption.
Rounding	Headwinds, the Medicare outpatient shift and the REH payment are rounded to $0.01M so the model ties to the case study figures.

Inputs

Blue = input. Yellow = key assumption. Every value is an illustrative assumption unless the note cites a source.

Assumption	Value	Unit	Source or note
Baseline revenue, FY2026			
Inpatient and swing-bed revenue	$6.10	$M	*Illustrative assumption*
Emergency department revenue	$9.40	$M	*Illustrative assumption*
Outpatient and ancillary revenue	$13.70	$M	*Illustrative assumption*
Other operating revenue	$0.80	$M	*Illustrative assumption*
Baseline expenses, FY2026			
Salaries and benefits	$17.60	$M	*Illustrative assumption*
Supplies and pharmacy	$4.30	$M	*Illustrative assumption*
Purchased services and contracted physicians	$5.10	$M	*Illustrative assumption*
Depreciation and interest	$1.80	$M	*Illustrative assumption*
Other expenses	$2.60	$M	*Illustrative assumption*
Payer mix (share of net patient service revenue)			
Medicare	48.0%	%	*Illustrative assumption*
Medicaid	17.0%	%	*Illustrative assumption*
Commercial	27.0%	%	*Illustrative assumption*
Self-pay	8.0%	%	*Illustrative assumption; shares must sum to 100%*
Operations			
Full-time equivalents (FTEs)	212	FTEs	*Illustrative assumption*
ED visits per year	11,400	visits	*Illustrative assumption*
Inpatient admissions per year	580	admissions	*Illustrative assumption*
Average daily inpatient census	6.2	patients	*Illustrative assumption*
Federal policy headwinds			
Expansion adults as share of Medicaid revenue	35.0%	%	*Illustrative assumption*
Expansion adults losing coverage after work requirements	18.0%	%	*Illustrative assumption; work requirements due by Jan 1, 2027 under OBBBA*
Marketplace plans as share of commercial revenue	12.0%	%	*Illustrative assumption*
Marketplace enrollees who drop coverage	25.0%	%	*Illustrative assumption; enhanced premium tax credits expired Dec 31, 2025*
State supplemental Medicaid payment cut (share of Medicaid revenue)	6.0%	%	*Illustrative assumption; provider-tax reductions begin Oct 1, 2027*
Rural Emergency Hospital (REH) inputs			
REH monthly facility payment, CY2026	$295,051.54	$	*Source: CMS MLN Fact Sheet MLN2259384, Dec 2025 (after sequestration)*
Months of facility payment per year	12	months	*CMS pays in 12 monthly installments*
Medicare share of ED and outpatient revenue	48.0%	%	*Illustrative assumption*
Medicare outpatient payment shift under REH (OPPS + 5% vs cost-based)	6.0%	%	*Illustrative assumption; shift is a reduction*
Observation revenue retained after conversion	$0.90	$M	*Illustrative assumption*
Inpatient costs removed after conversion	$5.40	$M	*Illustrative assumption*
FTEs removed after conversion	38	FTEs	*Illustrative assumption*
Scenario C and operating inputs			
Net FTEs added in Scenario C	10	FTEs	*Illustrative assumption*
ED transfer rate, baseline	6.0%	%	*Illustrative assumption*
ED transfer rate, target	4.5%	%	*Illustrative target*
Days in accounts receivable, baseline	58	days	*Illustrative assumption*
Days in accounts receivable, target	46	days	*Illustrative target*
Eligible out-of-network emergency claims per year	340	claims	*Illustrative assumption*
Average incremental recovery per claim	$1,030	$	*Illustrative assumption*
REH review trigger: operating margin below	(3.0%)	%	*Board policy trigger (case study Section 3, Part D)*
Stress case: share of initiative revenue delivered	50%	%	*Illustrative stress case*
Stress case: Medicare outpatient shift under REH	12.0%	%	*Illustrative stress case*

Check: payer mix sums to 100% OK

Baseline: FY2026

$ millions. Links to Inputs.

Revenue ($M)	FY2026
Inpatient and swing-bed	$6.10
Emergency department	$9.40
Outpatient and ancillary	$13.70
Net patient service revenue	**$29.20**
Other operating revenue	$0.80
Total operating revenue	**$30.00**

Expenses ($M)	FY2026
Salaries and benefits	$17.60
Supplies and pharmacy	$4.30
Purchased services and contracted physicians	$5.10
Depreciation and interest	$1.80
Other	$2.60
Total operating expenses	**$31.40**
Operating result ($M)	**($1.40)**
Operating margin	**(4.7%)**

Payer mix	Share of net patient revenue	Revenue ($M)
Medicare	48.0%	$14.00
Medicaid	17.0%	$5.00
Commercial	27.0%	$7.90
Self-pay	8.0%	$2.30
Total	**100.0%**	**$29.20**
Check: payer revenue ties to net patient service revenue	OK	

Payer revenue is rounded to $0.1M to match the case study.

Federal policy headwinds, FY2028

$ millions. Each impact is rounded to $0.01M.

Headwind	Revenue exposed ($M)	Rate	Annual revenue impact ($M)	Starts
Medicaid work requirements	$1.75	18.0%	($0.32)	Jan 1, 2027
Marketplace subsidy expiration	$0.95	25.0%	($0.24)	Jan 1, 2026
State Medicaid financing pressure	$5.00	6.0%	($0.30)	FY2028
Total			**($0.86)**	
Headwinds as share of operating revenue			2.9%	

Loss by payer	Share of net patient revenue	Revenue lost ($M)	Share of loss
Medicare	48.0%	-	-
Medicaid	17.0%	$0.62	72.1%
Commercial	27.0%	$0.24	27.9%
Self-pay	8.0%	-	-
Total	**100.0%**	**$0.86**	**100.0%**

Medicare and self-pay carry no modeled loss (assumption, blue zeros).

Scenario C: six transformation initiatives

Run-rate by FY2028, $ millions. Blue = input.

#	Initiative	Type	Gain or savings ($M)	New annual cost ($M)	Net ($M)	AI tooling inside new cost ($M)	AI-tooled?	Policy hook
1	Primary care and behavioral health clinic expansion	Revenue	$1.20	$0.80	$0.40	$0.10	Y	RHTP access and workforce priorities
2	Tele-emergency and telepsychiatry	Revenue	$0.50	$0.40	$0.10	$0.10	Y	RHTP technology priorities
3	Swing-bed utilization program	Revenue	$0.60	$0.10	$0.50	-	N	CAH cost-based payment
4	Revenue cycle upgrade and No Surprises Act dispute desk	Revenue	$0.35	$0.20	$0.15	$0.10	Y	May 2026 IDR rule: $15 fee, 50-line batching
5	Supply chain and purchased-services renegotiation	Savings	$0.80	-	$0.80	-	N	Margin protection
6	Retention program and float pool to cut agency labor	Savings	$0.60	-	$0.60	-	N	Workforce stability
	Total		**$4.05**	**$1.50**	**$2.55**	**$0.30**		

Summary	Value
Revenue-type gains ($M)	$2.65
Cost savings ($M)	$1.40
New annual cost ($M)	$1.50
Gross gain in AI-tooled initiatives ($M)	$2.05
AI-tooled share of gross gain	50.6%
AI tooling as share of new cost	20.0%

Operating checks	Value
ED transfers per year, baseline	684
ED transfers per year, target	513
Transfers avoided per year	171
One-time cash from fewer A/R days ($M)	$0.96
Dispute-desk recoveries from claim count ($M)	$0.35
Difference vs initiative 4 gain ($M)	($0.000)
Expansion-adult Medicaid revenue ($M)	$1.75
Disenrollment rate with outreach (option)	14.0%
Coverage-outreach benefit ($M, outside base plan)	$0.07

Type must be Revenue or Savings; SUMIF formulas use it. AI tooling and AI-tooled flags are assumptions (initiatives 5 and 6 carry no separate AI budget).

FY2028 scenario comparison

$ millions, steady state. Headwinds apply to all three; expenses held at FY2026 dollars.

Line ($M)	A: Hold as CAH	B: Convert to REH	C: CAH transformation
Baseline operating revenue	$30.00	$30.00	$30.00
Federal headwinds	($0.86)	($0.86)	($0.86)
Inpatient and swing revenue lost	-	($6.10)	-
Observation stays retained	-	$0.90	-
Medicare outpatient shift to OPPS	-	($0.67)	-
REH monthly facility payment	-	$3.54	-
Initiative revenue gains	-	-	$2.65
Operating revenue	**$29.14**	**$26.81**	**$31.79**
Baseline operating expenses	$31.40	$31.40	$31.40
Inpatient costs removed	-	($5.40)	-
New program costs	-	-	$1.50
Cost savings	-	-	($1.40)
Operating expenses	**$31.40**	**$26.00**	**$31.50**
Operating result ($M)	**($2.26)**	**$0.81**	**$0.29**
Operating margin	**(7.8%)**	**3.0%**	**0.9%**
Below REH trigger?	Breach	OK	OK
FTEs	212	174	222

Decision metrics	
Net revenue effect of REH conversion, before headwinds ($M)	($2.33)
REH result after revenue changes, before cost cuts ($M)	($4.59)
Scenario C result improvement vs Scenario A ($M)	$2.55

Tie-out to case study	A	B	C
Case-study margin (typed from the case study)	(7.8%)	3.0%	0.9%
Model margin, rounded to 0.1%	(7.8%)	3.0%	0.9%
Status	OK	OK	OK

Sensitivity and stress cases

Scenario C operating result, $ millions. Each assumption moves alone. Blue = input.

Scenario C base operating result ($M) **$0.29**

Assumption	Type	Base	Worse case	Better case	Result, worse ($M)	Result, better ($M)	Swing ($M)
Initiative revenue uplift (share of plan)	Execution	100%	50%	150%	($1.04)	$1.61	$2.65
Cost savings realized (share of plan)	Execution	100%	50%	150%	($0.41)	$0.99	$1.40
State Medicaid financing cut	Policy	6.0%	12.0%	-	($0.01)	$0.59	$0.60
New program costs (share of plan)	Execution	100%	120%	80%	($0.01)	$0.59	$0.60
Medicaid expansion disenrollment	Policy	18.0%	36.0%	9.0%	($0.03)	$0.45	$0.47
Marketplace attrition	Policy	25.0%	50.0%	12.5%	$0.05	$0.41	$0.36

Total swing, execution assumptions ($M) **$4.65**

Total swing, federal policy assumptions ($M) **$1.43**

Stress case	Operating revenue ($M)	Operating expenses ($M)	Result ($M)	Margin	Below REH trigger?
Scenario C: initiatives deliver only part of their revenue	$30.47	$31.50	($1.04)	(3.4%)	Breach
Scenario B: larger Medicare outpatient shift under REH	$26.14	$26.00	$0.14	0.6%	OK

Margin by scenario (feeds the comparison chart)	Case	Margin
A: Hold as CAH	Base	(7.8%)
B: Convert to REH	Base	3.0%
B: Convert to REH	Stress	0.6%
C: CAH transformation	Base	0.9%
C: CAH transformation	Stress	(3.4%)

AI tooling: cost against attributable benefit

$ millions a year. Attribution shares are illustrative assumptions, not evidence.

Low-case benefit realization	50%
High-case benefit realization	150%

AI tool	Linked initiative	Annual tooling cost ($M)	Linked initiative gain ($M)	Share attributable to AI	Base benefit ($M)	Low benefit ($M)	High benefit ($M)	Base return (x cost)
Revenue-cycle and dispute-claim AI	4	$0.10	$0.35	50%	$0.18	$0.09	$0.26	1.75x
Tele-ED decision support	2	$0.10	$0.50	25%	$0.13	$0.06	$0.19	1.25x
Ambient clinic documentation	1	$0.10	$1.20	10%	$0.12	$0.06	$0.18	1.20x
Total		**$0.30**			**$0.42**	**$0.21**	**$0.63**	**1.40x**

Break-even benefit realization (tooling cost / base benefit)	**71.4%**
Low-case return (x cost)	0.70x
High-case return (x cost)	2.10x

Supply-chain analytics (initiative 5) has no separate tooling budget and is excluded. Coverage-outreach benefit is on the Initiatives sheet.